FOR MEDICAL APPOINTMENTS

SETD5 Syndrome: *Medical Appointment Guide*

setd5syndrome.live

WHAT IS SETD5 SYNDROME?

SETD5 Syndrome is a rare genetic condition caused by a variant (mutation or deletion) in the SETD5 gene on chromosome 3 (3p25.3). When one copy is missing or not working, gene regulation is disrupted, resulting in haploinsufficiency. Most cases arise de novo (not inherited), but it can be inherited. The condition affects each person differently; no two individuals share the exact same presentation.

FEATURES REPORTED IN SETD5 SYNDROME

Not every individual will have all features listed. Severity varies widely, even among people with the same variant type.

NEURODEVELOPMENTAL & BEHAVIORAL

- Intellectual disability (mild to moderate; some borderline or near-typical)
- Global developmental delay
- Speech and language issues
- Some individuals are minimally verbal or nonverbal
- ADHD features: inattention, hyperactivity, impulsivity
- Autism spectrum disorder features in some
- Anxiety; behavioral rigidity; difficulty with transitions
- Sensory sensitivities; repetitive movements or behaviors
- Working memory and short-term memory difficulties

NEUROLOGICAL, PHYSICAL & GROWTH

- Hypotonia (low muscle tone); gait abnormalities including toe-walking
- Coordination difficulties; motor development delays
- Epilepsy or seizures in some; EEG abnormalities in some without clinical seizures
- Sleep disturbances common
- Feeding difficulties in infancy; failure to thrive in some; short stature
- Gastroesophageal reflux in some
- Congenital heart defects reported in some
- Eye and vision anomalies: strabismus, nystagmus, ptosis; refractive errors
- Hearing impairment (temporary or permanent) in some
- Scoliosis, kyphosis, joint laxity; reduced bone mineral density in some
- Cryptorchidism and/or hypospadias in some males

FOR THIS MEDICAL APPOINTMENT

Questions drawn from published research, clinician observations, and experiences reported by families across the broader SETD5 Syndrome community.

SUGGESTED EVALUATIONS & REFERRALS

- Should we pursue a genetics consultation if we haven't already?
- Given the risk of seizures, should we see a neurologist? Would an EEG or MRI be appropriate?
- Should we have cardiology, ophthalmology, and audiology screenings?
- Should we see an orthopedic specialist to monitor the spine and bone mineral density?
- How should we monitor growth? Would a developmental pediatrician help coordinate care?

QUESTIONS TO RAISE WITH YOUR PROVIDER

- How does SETD5 Syndrome relate to what we're currently seeing behaviorally?
- Are there genetic counseling resources for our extended family?
- What should we watch for as our child gets older?
- Can you help us understand how to access early intervention or therapy services?
- Are there research registries we should consider, such as Simons Searchlight?

GENE	INHERITANCE	PREVALENCE	OMIM	REGISTRY
SETD5 (chromosome 3p25.3)	Typically de novo; can be inherited (AD)	Fewer than 100 in published literature; ~1,100 in patient community	#615761	Simons Searchlight

More information, plain-language research summaries, and a medical terms guide are available at setd5syndrome.live. This document is for informational purposes only and does not constitute medical advice. Always consult your child's medical team for guidance specific to your situation. Prepared by setd5syndrome.live — a family-created resource.

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